

CliniCause: Constructing Reusable Causal-Analysis Datasets from Irregular ICU Records

Anonymous submission

Abstract

1 Introduction

Planned content: problem, resource-level contribution, scope, and bounded contribution summary.

2 Related Work

Planned content: compact gap-led synthesis using only verified bibliography entries.

3 CliniCause Resource and Pipeline

3.1 Source Cohorts and Preprocessing

Planned content: cohort definitions, analysis units, preprocessing, and split boundaries.

3.2 Proxy Construction

Planned content: rule source, predictive sources, aggregation, and proxy semantics.

3.3 Causal Estimation and Validation

Planned content: estimand, estimator roles, validation contract, and diagnostic boundaries.

4 Evaluation

4.1 Experimental Setup

Planned content: checked archive, selection rule, metrics, sampling conditions, and qualifications.

4.2 Predictive Results

Planned content: result-led comparison tied directly to Table 1.

4.3 Causal Results

Planned content: checked central pattern, named exceptions, and bounded robustness interpretation.

5 Discussion and Limitations

Planned content: bounded interpretation, explicit failure modes, provenance limits, and release state.

6 Conclusion

Planned content: concise resource-level conclusion with no new claims.

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Figure 1 placeholder: end-to-end CliniCause resource and pipeline.

Figure 1: Planned Figure 1: CliniCause resource and pipeline.

Table 1 placeholder: dataset scope and archived predictive test metrics.

Table 1: Planned Table 1: dataset scope and archived predictive results.

Figure 2 placeholder: cross-estimator directional agreement for 19 dataset–exposure combinations.

Figure 2: Planned Figure 2: cross-estimator directional agreement.

Table 2 placeholder: compact CATE and directional-concordance summary.

Table 2: Planned Table 2: causal-estimation and directional-concordance summary.

References